



POLICY

Medication: Prescribing and Supply of Stimulants

Scope (Worker):	NMHS Mental Health Staff: Clinical
Scope (Area):	NMHS Mental Health: All Areas

1. Aim

To ensure stimulants are prescribed in accordance with evidence-based practice and legislative requirements.

2. Background

The [WA Medicines and Poisons Regulations 2016](#) requires a medical practitioner to obtain authorisation from the Commissioner of Health before prescribing stimulant medication. Applications for authorisations to prescribe can be found at [WA Schedule 8 Medicines - Stimulant Medicines](#).

The Health Department allows hospitals to continue treatment with these medications for inpatients in consultation with the patient's authorised Stimulant Prescriber.

Inpatient commencement of treatment with stimulant medication must comply with the requirements for authorisation.

Consideration needs to be given to continuation of treatment in the community after discharge as the community prescriber must also comply with the requirements of the [WA Schedule 8 Medicines Prescribing Code](#).

3. Risk

Failure to comply with this policy may result in non-compliance with legislative requirements and compromised patient care.

Non-compliance with this policy will impact on the following risk categories:

Governance & Accountability	☒	Policy & Legislative Compliance	☒
		Quality of Patient Care	☒

4. Definitions

Stimulants are a class of medicines which stimulate the central nervous system (CNS) by increasing the activity of certain chemicals in the brain.

Stimulants include all formulations of:

- dexamfetamine
- lisdexamfetamine (Vyvanse®)

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- methylphenidate (Ritalin®, Ritalin LA®, Artige®, Concerta®).

5. Principles

Treatment with stimulants must only be commenced by a psychiatrist registered as an authorised Stimulant Prescriber with the [WA Schedule 8 Medicines - Stimulant Medicines](#) and must comply with the requirements of the [WA Schedule 8 Medicines Prescribing Code](#).

For continued stimulant medicines treatment post discharge the medical practitioner must contact the community treating psychiatrist to ensure that the community treating psychiatrist is registered or can become registered with the Stimulant Regulatory Scheme and is able to continue the care of that patient on the stimulant medication.

Where a patient is admitted to hospital and the treating team considers it appropriate to continue existing stimulant treatment, this may continue provided:

- the patient is already notified or authorised;
- the prescribing is consistent with the notification or authorisation;
- the practitioner is satisfied it is safe to prescribe stimulants; and
- treatment does not exceed three months, as per legislation.

The treating medical practitioner or clinical pharmacist will liaise with the patient's authorised Stimulant Prescriber regarding continued treatment and the medical practitioner should contact the Department of Health S8 Prescriber Information Service to obtain a stimulant notification history and S8 prescribing history.

Outside these conditions, WA Health authorisation is required.

Significant changes to stimulant treatment such as changing from one stimulant medication to another may only be made by a psychiatrist registered with the [WA Schedule 8 Medicines - Stimulant Medicines](#).

6. Compliance and Evaluation

NMHS MH Pharmacy will evaluate compliance with this policy via 3-monthly report to NMHS MH Drug and Therapeutics Committee (DTC).

- 100% of inpatient prescriptions for stimulant medicines will be discussed with the patient's authorised Stimulant Prescriber.
- 100% of community prescriptions for stimulants will be written by an authorised Stimulant Prescriber.

References

[WA Schedule 8 Medicines Prescribing Code](#)
[WA Schedule 8 Medicines - Stimulant Medicines](#)
[WA Medicines and Poisons Regulations 2016](#)
[WA Medicines and Poisons Act 2014](#)
[National Safety & Quality Health Standards Ed 2 2017](#)

Policy Sponsor	NMHS MH Chief Pharmacist				
Policy Contact	NMHS MH Chief Pharmacist				
Date First Issued:	30/12/2015	Last Reviewed:	16/01/2024	Review Date:	16/01/2027
Approved by:	NMHS MH Drug and Therapeutics Committee			Date:	05/12/2023
Endorsed by:	MHPHDS Executive Director			Date:	23/01/2024
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 8: Governance, leadership and management		☐ Std 9: Integration ☒ Std 10: Delivery of Care		
Chief Psychiatrist's Standards for Clinical Care:	Risk Assessment and Management, Transfer of Care				
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Document Version Control

Date	Version	Author	Notes
30/12/2015	V1.0	Chief Pharmacist	Patrick Marwick
24/07/2017			MHEG, Ros Elmes, out of session email
24/05/2018	V2.1	A/Policy Officer	Updated hyperlinks. Review date remains June 2020
17/07/2020	V3.0	NMHS MH DTC	Out of session endorsement email
14/10/2020		NMHS MH MHMC	Out of session endorsement email
30/10/2020		Exec Director, MHPHDS	Signed submission for in policy folder on W drive
05/11/2020	V3.0	MHPHDS Policy Officer	Updated template & headings; retitled; content aligned with DoH S8 med protocols
16/01/2024	V4.0	Chief Pharmacist	Scheduled review; new stimulants information added, template and hyperlinks undated

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative

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